Medical Emoji Briefing

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A Briefing for the Council of Medical Specialty Societies

Prepared for Clifford A. Hudis, MD — President, CMSS · CEO, ASCO Prepared by Shuhan He, MD, on behalf of the Medical Emoji Project · June 18, 2026 medicalemoji.org

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1. The opportunity in one paragraph

Emoji are the most widely used visual language in human history — present on virtually every phone, in every messaging app, in patient texts, public-health campaigns, and increasingly in clinical and educational materials. Yet the medical vocabulary is nearly empty. There is no kidney, no liver, no stomach, no EKG. Our team changed that once: in 2020 we helped add the **anatomical heart** and **lungs** to the global Unicode Standard. We are now pursuing the next slate of clinically essential symbols and asking CMSS to lend the credibility of organized medicine to the effort.

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2. Track record: this has been done before

- **2020 — Anatomical heart (🫀) and lungs (🫁) approved** and shipped in Unicode Emoji 13.0. These were the first anatomically accurate internal-organ emoji, and they now appear on Apple, Google, Samsung, and Microsoft platforms worldwide.
- Led by a physician team out of Massachusetts General Hospital, working through the official Unicode Consortium proposal process.
- Coverage and recognition:
    - **The Boston Globe** — "This Mass General doctor helped get two new medical emojis approved." https://www.bostonglobe.com/2020/02/04/metro/this-mass-general-doctor-helped-get-two-new-medical-emojis-approved/
    - **The Verge** — "Doctors are fighting for better medical emoji." https://www.theverge.com/2021/9/13/22665002/doctors-medical-emoji-organs-health
    - **Harvard Medical School** — "Emoji Power." https://hms.harvard.edu/news/emoji-power

    - **WCVB Boston**, **Healio**, and others.

This is not a concept pitch. It is an extension of a proven, peer-recognized success.

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3. The evidence base

- **JAMA** — the clinical rationale for medical emoji, published in the *Journal of the American Medical Association*. https://jamanetwork.com/journals/jama/article-abstract/2783847
- **Hepatology** (AASLD journal) — companion letter supporting the liver concept. https://aasldpubs.onlinelibrary.wiley.com/doi/abs/10.1002/hep.32261
- **The Spine Journal** — companion article supporting the spine concept. https://www.thespinejournalonline.com/article/S1529-9430(22)00070-5/fulltext

The core argument across these is consistent: standardized medical symbols improve clarity in patient communication, support health literacy across language barriers, and give clinicians and public-health communicators a shared, universally rendered visual shorthand.

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4. The concepts we are pursuing

Each is tied to a high-burden condition and a clear communication use case.

Concept	Why it matters (illustrative burden)
Kidney	Chronic kidney disease affects ~10% of the global population; ~1.3M deaths/year.
Liver	Liver disease causes ~2M deaths/year; ~25% of US adults have NAFLD.
Stomach	GERD alone: ~$18B direct and ~$75B indirect annual costs.
Spine	Low back pain is the #1 cause of years lived with disability worldwide.
White blood cell	Immunity, infection, and pandemic communication — from sepsis to COVID-19.
EKG	Cardiovascular disease kills one American every ~36 seconds.

We are also maintaining a broader reference set (intestine, blood bag, pill pack, weight scale) and a comprehensive medical-emoji site so the field has a single, authoritative home.

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5. Organized medicine is already behind this

We hold formal letters of support from, among others:

- **American College of Emergency Physicians (ACEP)** — a *project-wide* endorsement naming all ten concepts at once.
- **American Gastroenterological Association (AGA)** — liver, stomach, intestine.
- **American Society for Gastrointestinal Endoscopy (ASGE)** — liver, stomach, intestine.
- **Nephrology coalition** — American Society of Nephrology (ASN), International Society of Nephrology (ISN), National Kidney Foundation (NKF), Renal Physicians Association, Canadian Society of Nephrology, KDIGO, NephJC, ASDIN, American Association of Kidney Patients, and more — all for the kidney.
- Journal-level support from **Hepatology** and **The Spine Journal**.

What we do not yet have is a voice that speaks for the breadth of organized medicine across specialties at once. That is precisely what CMSS is.

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6. How Unicode works — and what a CMSS letter actually does

Unicode approves new emoji through a formal proposal process with two pillars:

1. **Evidence of expected high-frequency, real-world usage** (search/trend/usage data). We build this ourselves; society letters are explicitly *not* counted as frequency evidence by Unicode.
2. **Credibility and clarity** — that the symbol is well-defined, distinct, and that recognized authorities consider it meaningful.

A CMSS letter does its work on the second pillar and beyond it: it signals to the Unicode Emoji Subcommittee that the most authoritative body in American organized medicine considers a standardized clinical emoji set important and legitimate. Just as importantly, it positions CMSS — and Unicode — for a relationship rather than a transaction: a standing channel through which the house of medicine can advise Unicode on clinically accurate, useful symbols over time. That partnership framing is the longer game, and it is one only a council-level body can credibly open.

To be candid about scope: a single letter will not, by itself, guarantee any specific approval. Its value is credibility, signaling, and the door it opens. The frequency evidence and proposal drafting remain our job.

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7. Why it matters

- **Patient communication & health literacy.** A kidney or liver symbol that renders identically on every device gives clinicians, educators, and public-health campaigns a shared visual anchor — across languages and literacy levels.
- **Equity and reach.** Emoji are free, ubiquitous, and cross-linguistic. A standardized medical set is public-health infrastructure that reaches populations text often cannot.
- **Stewardship.** Medical symbols are entering global culture whether or not physicians are at the table. Organized medicine should help define them accurately rather than inherit whatever appears.

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8. What we are asking of CMSS

One Council-level letter that:

1. Affirms the clinical-communication and health-literacy value of a standardized set of medical emoji; and
2. Encourages the Unicode Consortium to consider these concepts and to engage organized medicine as an ongoing partner on clinically accurate symbols.

This does **not** ask CMSS to draft proposals, fund the work, or take on ongoing obligations. We handle the Unicode submissions, evidence, and artwork.

A model paragraph CMSS may adapt freely:

> *The Council of Medical Specialty Societies, representing [N] medical specialty societies and more than [N] physicians, supports efforts to expand the standardized vocabulary of medical emoji within the Unicode Standard. Clear, universally rendered visual symbols for common clinical concepts — such as the kidney, liver, stomach, spine, white blood cell, and electrocardiogram — can strengthen patient communication, health literacy, and public-health messaging across languages and platforms. We encourage the*

> *Unicode Consortium to give these concepts full consideration and to engage organized medicine as a partner in ensuring future clinical symbols are accurate and useful.*

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9. Next steps

We are happy to:

- Present to the relevant CMSS committee or board, in person or by video.
- Provide the full library of existing support letters, JAMA and specialty-journal publications, and draft Unicode proposals for review.
- Move entirely at the pace and through the process the Council prefers.

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